



Diagnostic Imaging Consultation

X-RAY . CBCT . MRI . CT . ULTRASOUND

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PATIENT NAME:
DATE OF BIRTH:
GENDER:
DATE OF EXAM:

STUDY PERFORMED AT:

REFERRING PHYSICIAN:

DIAGNOSTIC IMAGING REPORT

MR IMAGING – LUMBAR SPINE WITHOUT CONTRAST

INDICATION: Low back pain after injury, left leg pain

COMPARISON: None.

TECHNIQUE: MRI images were obtained in long- and short-axis, fat-weighted and fluid-weighted, without the infusion of contrast.

FINDINGS:

The lumbar spine demonstrates normal lordotic curvature and alignment. Vertebral body heights and marrow signal are within normal limits. Conus medullaris terminates near the L1-L2 level.

L1-L2: Mild facet hypertrophy. No disc herniation or stenosis.

L2-L3: Mild facet hypertrophy. Trace disc displacement. Mild foraminal stenosis.

L3-L4: Annular tear and broad-based left paracentral disc protrusion, best seen on sagittal T2 series 2 image 8. This measures about 3.7 mm AP and results in moderate narrowing of the left lateral recess. Mild to moderate facet hypertrophy also contributes to narrowing of the lateral recess. There is associated ligamentum flavum hypertrophy. Mild to moderate bilateral foraminal stenosis.

L4-L5: Moderate facet hypertrophy. Shallow disc displacement. Mild to moderate foraminal stenosis.

L5-S1: No disc herniation or stenosis.

Coronal STIR series of the pelvis demonstrates no marrow edema in the bony pelvis or hips. There is normal alignment. Visualized intrapelvic contents are grossly unremarkable. Visualized surrounding soft tissues are unremarkable.

IMPRESSION:

1. Left paracentral annular tear and broad-based disc protrusion at L3-L4 measuring maximally 3.7 mm AP, resulting in moderate left lateral recess stenosis.
2. Facet hypertrophy throughout the lumbar spine, most notable at L3-L4 and L4-L5 where results in mild to moderate foraminal stenosis.

Electronically signed by:

September 29, 2023